

Risk Summary

Preeclampsia

Medium Score: 0.6395082154771051

Model Interpretation: Risk-based triage only (not diagnosis).

Macrosomia

Medium Score: 0.5943816940585895

Model Interpretation: Risk-based coaching support.

Summary: Medium risk for both preeclampsia and macrosomia. Elevated systolic blood pressure with proteinuria noted, requiring urgent medical review. Fasting glucose is borderline, with high pre-pregnancy BMI, suggesting ongoing need for lifestyle guidance despite favorable HbA1c.

Patient Snapshot (Recent Inputs)

| Parameter              | Value                                        | Notes                                                      |
|------------------------|----------------------------------------------|------------------------------------------------------------|
| Blood Pressure         | 142/79 mmHg                                  | Hypertensive threshold ≥140/90, severe ≥160/110.           |
| Proteinuria (dipstick) | 1                                            | ≥1+ supports PE suspicion in context of HTN.               |
| Symptoms               | Headache=0, Vision=0, RUQ Pain=0, Swelling=0 | Severe features increase escalation priority.              |
| Glucose                | Fasting=99 mg/dL<br>1hr PP=126 mg/dL         | Targets typically fasting <95, 1hr <140 (pregnancy goals). |
| HbA1c                  | 5.6                                          | Higher HbA1c indicates sustained glycemic elevation.       |
| Maternal Context       | Age=38<br>BMI=42.4<br>GDM=0                  | Risk modifiers for fetal overgrowth + metabolic risk.      |
| Activity / Sleep       | Post-meal walk=29 min<br>Sleep=6.1 hrs       | Lifestyle factors influence glycemic response.             |
| Food Log               | paneer salad                                 | Used for dietary correlation patterns.                     |

Reason for Alert

Action Requested: ALERT\_DOCTOR

Clinical Rationale: Patient TEST\_MED\_MED, G32W. Preeclampsia risk tier: medium, Macrosomia risk tier: medium. Key vitals: Systolic BP 142 mmHg, Diastolic BP 79 mmHg, Proteinuria dipstick 1. Fasting Glucose 99 mg/dL, Pre-pregnancy BMI 42.4 kg/m². No severe symptoms reported. Elevated systolic BP and proteinuria at 32 weeks gestation warrant clinical evaluation for hypertensive disorders of pregnancy, as per ACOG guidelines. Please review.

Suggested Next Steps

- Review trend: BP, symptoms, proteinuria, glucose readings
- Consider confirmatory tests (protein/creatinine ratio, LFTs, platelets) if clinically indicated
- Consider GDM diet review or insulin adjustment if glucose targets exceeded
- Provide patient education + return precautions

